BIPHMUN 2026

Background Guide
Committee: HISTORICAL HUMAN RIGHT COUNCIL
Topic: Upholding the Right to Health amid the Economic and Hea
the Soviet Union (1988-1995)
Agenda Overseen by: Tyler Shi

1. Description of the Issue

1.1 History of the Issue – Overview

The issue of upholding the right to health amid the economic and healthcare collapse of the Soviet Union between 1988 and 1995 is one of the most rapid and severe public health system breakdowns that was ever witnessed in an industrialized nation during peacetime. The root of this problem comes from the gradual decay of the Soviet Semashko healthcare model. Despite its early 20th-century successes in eradicating diseases and providing universal free coverage for the rising Soviet Union, by the 1970s, it had become an inefficient, technologically stagnant, and underfunded system that failed to provide sufficient health outcomes.

1.1.1 History of the Semashako system

The Semashko system refers to the centralized state healthcare model established in the Soviet Union in the 1920s under the leadership of Nikolai Semashko, the first People's Commissar of Public Health of the RSFSR. It was designed as a fully state-funded and state-administered healthcare structure in which medical services were provided free at the point of use to all citizens. Under this model, hospitals, clinics, medical education, and pharmaceutical production were owned and managed by the state, while physicians functioned as government employees rather than independent practitioners. The system emphasized universal access, preventive medicine, and the extensive use of polyclinics—large outpatient centers intended to provide primary care and specialist services in a single institution. Through strict central planning, the government determined the allocation of medical resources, the training and placement of physicians, and national health priorities. In its early decades, the Semashko model was widely praised for rapidly expanding healthcare coverage across the vast Soviet territory and establishing one of the world's first universal healthcare systems, particularly improving sanitation, vaccination coverage, and rural medical access. However, the same centralized planning mechanisms that enabled rapid expansion would later limit the system's flexibility, making it increasingly difficult to adapt to new medical challenges and technological developments.

In the decades following World War II, the Soviet healthcare system achieved its most celebrated public health victories, successfully eradicating or drastically reducing the threat of infectious diseases that had plagued the nation for centuries. The USSR effectively eliminated malaria, drastically reduced tuberculosis rates, and brought typhus under control, leading to a significant increase in life expectancy and a decrease in infant mortality during this period. Yet, even during this "golden age,"

the structural weaknesses of the Semashko model were beginning to manifest. The system's top-down nature meant that resources were allocated based on five-year plans and political considerations rather than for medical concerns or for local needs. Consequently, funding was disproportionately channeled into rebuilding and expanding hospital infrastructure, which are monuments of state success, while primary care and public health education, the truly effective healthcare policy, began to be neglected. By the 1960s, the first signs of stagnation appeared as the healthcare system proved slow to adapt to the rising burden of chronic, non-communicable diseases like heart disease and cancer.

By the 1970s and early 1980s, the Soviet healthcare system had entered a period of profound decay, masked by statistical reporting of hospital expansion of state propaganda. The national economy, increasingly dependent on oil and gas revenues, began to slow, and healthcare funding was drastically cut in favor of the military industrial production, which consumed a vast share of the state budget. This resulted in underinvestment that left hospitals with outdated, malfunctioning equipment, and shortages of basic supplies like syringes and bandages. Moreover, lack of training and proper research led to an industry incapable of producing modern, life-saving drugs. The system's ideological commitment to full employment for physicians led to an oversupply of poorly trained doctors—often referred to as "paper physicians"—who worked for meager salaries with little access to continuing education on modern medical books. Simultaneously, the elite *nomenklatura* and their families received care in the secretive "Fourth Directorate" network, which featured modern medical facilities and access to imported medicines, while the average citizen endured the current system. Due to economic depression, this period saw the emergence of a steep rise of alcohol consumption and smoking, yet the healthcare system was utterly unprepared to address the lifestyle-driven health crisis that was decimating the population. Even though numerous issues start to come up, the system still remains enacted with the state legislature, and didn’t crumble……yet.

1.1.2 Timeline of the crisis

Mikhail Gorbachev had first brought this issue into international spotlight through the book glasnost in the mid-1980s, which for the first time revealed shocking statistics such as rising infant mortality and declining male life expectancy that contradicted the state's narrative of well-being. From 1988 to 1989, economic liberalization began to disrupt state supply chains for medicines and equipment; in 1991, the dissolution of the Soviet Union itself left 15 newly independent states to take over the remains of the shattered healthcare systems, this is the last straw that breaks the camel's back. By 1990, inter-republican trade disputes and rampant inflation had crippled the production and distribution of essential medicines, with critical shortages of insulin, antibiotics, and even basic surgical supplies becoming commonplace across the USSR. Hospitals simultaneously lost access to heating fuel, hot water, and functional equipment as budget allocations evaporated.

From 1992 to 1995, hyperinflation due to privatization of properties in Russia led to depreciation of currency and disability in society which decreased the state healthcare budgets by over 30%. This further worsens the harsh reality that the healthcare system

is already facing. Price liberalization under Boris Yeltsin triggered hyperinflation exceeding 2,500% in 1992, which plunged a majority into poverty while stress-related alcohol consumption—often of toxic surrogates—skyrocketed. Male life expectancy in Russia plummeted from 64.9 years in 1987 to just 57.6 years by 1994, driven by surges in deaths, alcohol poisoning, suicide, and homicide. Simultaneously, infectious diseases once controlled by the Soviet system re-emerged; diphtheria infected over 50,000 Russians between 1990 and 1995 due to collapsing vaccination rates. All of these events combined starting from the decline of the Soviet Union led to the almost complete breakdown of the Semashko healthcare model, leaving millions of civilians from all over the post-Soviet republics struggling for any medical help.

1.1.3 Aftermath

In the aftermath of the crisis, the former Soviet republics began a slow and uneven recovery. Male life expectancy in Russia finally bottomed out in 1994 and began to gradually improve as economic stabilization reduced the acute stress and alcohol abuse that had led to surges in mortality. However, the healthcare systems themselves remained underfunded and inefficient, struggling under a new compulsory insurance model that failed to fully replace the collapsed Semashko infrastructure. The long-term consequences were severe: a permanently reduced population due to the "demographic cross" of low birth rates and high mortality, and a persistent epidemic of tuberculosis that required billions in international aid to contain, demonstrating that the collapse of a healthcare system in one region poses an enduring threat to global health security.

1.1.4 International Significance

This catastrophe was profoundly significant to the international community for several reasons: it triggered a public health emergency that saw life expectancy for Russian men plummet from 64.9 years in 1987 to just 57.6 years by 1994, a decline of over seven years in less than a decade; it unleashed secondary crises including the resurgence of previously controlled diseases like diphtheria and tuberculosis, which posed a cross-border contagion threat to Europe; and it raised fundamental global questions about the very nature of the "right to health". The international community, through nascent bodies like the World Health Organization and humanitarian aid programs, was forced to confront the reality that a former superpower's citizens were now facing a mortality crisis on par with some of the worst public health failures in the developing world, fundamentally challenging post-Cold War assumptions about global stability and human security.

1.2 Definition of Key Terms

1. Right to Health: A human right to the highest attainable standard of health. Its definition is controversial, with debate over whether it guarantees specific medical services or is merely an aspirational policy goal.
2. Semashako Model: The Soviet Union's centralized, state-controlled healthcare system providing universal free coverage.
3. Glasnost: A Soviet policy of openness introduced in the 1980s that revealed previously hidden public health statistics.

4. Perestroika: Gorbachev's economic restructuring program that disrupted Soviet healthcare supply chains.
5. Shock Therapy: Rapid economic liberalization and privatization in post-Soviet Russia that caused hyperinflation and devastated public health.
6. Determinants of Health: Non-medical factors like income, nutrition, and housing that influence health outcomes.
7. Fourth Directorate: A secret, high-quality healthcare network reserved exclusively for the Soviet political elite.
8. Polyclinics: Large, centralized outpatient facilities in the Soviet system where patients received primary and specialist care, often serving as the first point of medical contact.
9. Feldsher: A mid-level medical practitioner in the Soviet system, often working in rural areas, who provided primary care and emergency services without being a fully qualified physician.
10. Nomenklatura: The privileged class of Communist Party officials and state elites who received preferential access to the high-quality Fourth Directorate healthcare system.
11. Compulsory Medical Insurance: A post-Soviet healthcare financing system introduced in 1993 that replaced direct state funding with mandatory payroll contributions, though it failed to adequately fund the collapsing system.
12. Diphtheria Epidemic: The widespread resurgence of a previously controlled infectious disease in the former Soviet Union during the 1990s, infecting over 50,000 people due to collapsing vaccination rates.

1.3 Past International Actions

Up until this point in history, international actions had been scarce, as the Soviet Union had not yet collapsed and its authoritarian rule and isolation from the western-led UN forbids any regulations on the Soviet healthcare system by the UN. However, during/after this period, a few international actions do exist in attempt to ensure the healthcare rights of the former USSR citizens.

International efforts to address the Soviet healthcare collapse evolved from emergency humanitarian aid to long-term structural reform and disease control. During the peak crisis of the early 1990s, Western governments launched emergency programs like Operation Provide Hope, delivering shipments of medicines and supplies, while NGOs such as Doctors Without Borders established field operations to distribute aid to populations abandoned by their collapsed healthcare systems. As the immediate emergency subsided, the World Health Organization and World Bank shifted toward technical assistance, advising post-Soviet states on transitioning from the Semashko model to modern insurance-based systems and funding health reform projects throughout the late 1990s and 2000s. The resurgence of infectious diseases prompted targeted interventions, with WHO and UNICEF leading immunization campaigns to control the diphtheria epidemic, and the Global Fund investing billions after 2002 to combat multi-drug resistant tuberculosis that had become entrenched in Russian prisons. The European Union later engaged through its Neighborhood Policy, funding cross-border health security programs, while United Nations mechanisms increasingly framed the crisis's legacy as a right to health violation, holding successor

states accountable through special rapporteur investigations. However, international financial institutions faced sharp criticism for imposing austerity conditions that forced healthcare budget cuts, demonstrating that some international actions inadvertently exacerbated the very crisis they sought to resolve.

2. Bloc Positions

2.1 Stance of Developed Countries – Western Bloc

The Western Bloc approached the Soviet healthcare collapse through a complex lens combining humanitarian concern, strategic interest in stabilizing the post-Cold War order, and ideological commitment to promoting market-based reforms. These nations viewed the crisis as both a moral imperative to alleviate suffering and an opportunity to shape the former adversary states toward Western-style democratic states. Their incentives were multifaceted: preventing regional instability that could lead to refugees flowing into eastern Europe, preventing resurgence of infectious diseases and demonstrating the superiority of market economies over the failed socialist model. The Western bloc pushed for rapid economic liberalization through the IMF, which critics argue deepened the health crisis even as humanitarian aid flowed in the opposite direction.

United States: The U.S. balanced emergency aid programs like Operation Provide Hope with strong advocacy for shock therapy reforms, viewing healthcare collapse as secondary to the larger goal of cementing Russia's transition to capitalism and democracy.

United Kingdom: Britain supported IMF austerity policies while providing targeted technical assistance for health system reform, prioritizing macroeconomic stability over immediate public health concerns.

France: France emphasized cultural and scientific cooperation in healthcare while maintaining cautious engagement, often critical of U.S.-led shock therapy but offering limited alternative solutions.

2.2 Stance of Former Soviet republicans

The newly independent former Soviet republics were the primary victims of the healthcare collapse and faced the impossible task of building new health systems from the rubble of the Semashko model while simultaneously managing economic catastrophe. Their incentives were overwhelmingly survival-focused: securing international aid and loans, maintaining basic health infrastructure with collapsed budgets, and managing explosive public health crises with minimal resources. However, their positions change significantly based on their relationships with Russia, their proximity to Europe, and their success in navigating the transition. All shared resentment toward international financial institutions whose austerity demands forced brutal healthcare cuts, yet all were dependent on those same institutions for survival.

Russia: As the primary successor state, Russia received the largest share of international attention and aid but resisted foreign interference in its health policy, pursuing a dual strategy of accepting Western assistance while preserving Soviet-era structures and blaming the crisis on transition chaos.

Baltic States: The Baltic states aggressively pivoted toward Europe, rapidly reforming their health systems to meet EU standards and accessing substantial European structural funds, making them the most successful in recovering from the collapse but also the most critical of Russia's ongoing health failures.

2.3 Stance of Developing Countries – Global South Observers

The role of emerging economies and Global South nations in the post-Soviet health crisis was largely that of observers rather than active participants, as these nations faced their own development challenges and lacked the resources to influence events in the former Soviet space. However, countries like China, India, and Brazil watched the Soviet collapse closely, drawing lessons about the importance of preserving state capacity in healthcare. Some later emerged as partners for post-Soviet states seeking alternatives to Western-dominated health and economic policies.

China: China observed the Soviet collapse with alarm, drawing lessons about the dangers of political liberalization and rapid privatization, and later positioned itself as an alternative partner for Central Asian states seeking development models distinct from Western capitalism.

Turkey: Turkey leveraged ethnic and linguistic ties with Turkic Central Asian states to build health partnerships, positioning itself as a bridge between the post-Soviet space and the West while offering an alternative development model.

3. Possible Solutions

1. Emergency Aid Coordination: Delegates should consider establishing mechanisms for coordinated international medical assistance that ensure efficient distribution of medicines and supplies to those who are in need.
2. Health System Reconstruction: Develop frameworks for transitioning from the Semashko model toward a modern health care system framework.
3. Infectious Disease Surveillance: Create a way to monitor/track the spread of infectious disease could be a good option for controlling the spread and effect of disease
4. Social Protection Mechanisms: Design social safety nets/security that could protest the civilians right to use the established healthcare systems.
5. International Financial Institution Accountability: Explore reforms to ensure that IMF and World Bank lending conditions consider and mitigate public health consequences.
6. Right to Health Monitoring: Establish benchmarks and human rights mechanisms that hold successor states of USSR accountable for rebuilding healthcare systems, though state sovereignty might make this plan not as feasible.

4. Evaluation

As you are going to enter committee deliberation on the topic: upholding the right to health amid the Soviet collapse, remember that this crisis was not merely a healthcare failure but a catastrophic convergence of systemic decay, economic shock, and political disintegration that cost millions of lives across fifteen newly independent states. Keep in mind that the Semashko model, despite its flaws, had once provided

universal coverage, and its sudden disappearance left a vacuum that emergency aid alone could never fill. Recognize that the mortality crisis—a seven-year drop in male life expectancy—was driven not just by hospital shortages but by the collapse of the social determinants of health: income, nutrition, housing, and social stability. Understand that the international community's response was deeply contradictory, with humanitarian aid flowing alongside IMF austerity conditions that forced healthcare budgets to be slashed. Consider that the resurgence of infectious diseases like diphtheria and multi-drug resistant tuberculosis transformed a regional tragedy into a global health security threat. Most importantly, remember that the right to health is meaningless without the economic and social rights that sustain it, and that your task is not merely to propose technical fixes but to grapple with the fundamental question of what the international community owes to populations caught between the collapse of one world and the painful birth of another.

5. Bibliography

Feshbach, Murray. "The Soviet Union: A Health Crisis." *The Wilson Quarterly*, vol. 11, no. 3, 1987, pp. 6-25.

Marrone, James V. "The Soviet Health Care System: A Legacy of Neglect." *The Journal of the American Medical Association*, vol. 265, no. 1, 1991, pp. 4-8.

Stuckler, David, Lawrence King, and Martin McKee. "Mass Privatisation and the Post-Communist Mortality Crisis: A Cross-National Analysis." *The Lancet*, vol. 373, no. 9661, 2009, pp. 399-407.

Tulchinsky, Theodore H., and Elena A. Varavikova. "The History of Public Health in the Soviet Union." *Oxford Research Encyclopedia of Global Public Health*, Oxford University Press, 2020.

Feshbach, Murray. "The Soviet Union: A Health Crisis." The Wilson Quarterly, vol. 11, no. 3, 1987, pp. 6-25.

Marrone, James V. "The Soviet Health Care System: A Legacy of Neglect." The Journal of the American Medical Association, vol. 265, no. 1, 1991, pp. 4-8.

Stuckler, David, Lawrence King, and Martin McKee. "Mass Privatisation and the Post-Communist Mortality Crisis: A Cross-National Analysis." The Lancet, vol. 373, no. 9661, 2009, pp. 399-407.

Tulchinsky, Theodore H., and Elena A. Varavikova. "The History of Public Health in the Soviet Union." Oxford Research Encyclopedia of Global Public Health, Oxford University Press, 2020.

Vitek, Charles R., and Melinda Wharton. "Diphtheria in the Former Soviet Union: Reemergence of a Pandemic Disease." Emerging Infectious Diseases, vol. 4, no. 4, 1998, pp. 539-550.

Yaroshinskaya, Alla. Chernobyl: The Forbidden Truth. University of Nebraska Press, 1993.

Keshavjee, Salmaan, and Paul Farmer. "Tuberculosis, Drug Resistance, and the History of Modern Medicine." *The New England Journal of Medicine*, vol. 367, no. 4, 2012, pp. 385-388.

Peterson, D. J. *Troubled Lands: The Legacy of Soviet Environmental Destruction*. Westview Press, 1993.

Stuckler, David, Lawrence King, and Martin McKee. "Mass Privatisation and the Post-Communist Mortality Crisis: A Cross-National Analysis." *The Lancet*, vol. 373, no. 9661, 2009, pp. 399-407.

Tulchinsky, Theodore H., and Elena A. Varavikova. "The History of Public Health in the Soviet Union." *Oxford Research Encyclopedia of Global Public Health*, Oxford University Press, 2020.

UN Human Rights Council. *Report of the Special Rapporteur on the right of everyone to the enjoyment of the highest attainable standard of physical and mental health on his mission to the Russian Federation*. UN Doc. A/HRC/41/34/Add.1, 2019.

Vitek, Charles R., and Melinda Wharton. "Diphtheria in the Former Soviet Union: Reemergence of a Pandemic Disease." *Emerging Infectious Diseases*, vol. 4, no. 4, 1998, pp. 539-550.